

RED REPORT

What we know for *this* patient

- He had **localized prostate adenocarcinoma (Gleason 4+3, Grade Group 3)** treated with **radical prostatectomy, negative margins**, and has had **undetectable PSA for 12 months** (no biochemical recurrence).
- He now has **symptoms consistent with hypogonadism** plus **two low morning total testosterone measurements**, which meets usual biochemical confirmation requirements for testosterone deficiency. ([Endocrine Society](#))

Is TRT absolutely contraindicated after prostate cancer?

Not universally—but it depends on whose guideline you follow and how you frame the evidence.

- The **Endocrine Society (2018)** recommends **against testosterone therapy in men with prostate cancer** (a conservative, “avoid in known cancer” stance). ([Endocrine Society](#))
- Many **urology-focused discussions and guidance** acknowledge that, in practice, **TRT is sometimes offered to carefully selected men who appear disease-free after definitive treatment**, emphasizing **shared decision-making** and **close PSA monitoring**, because the available evidence (mostly observational) has **not shown higher short- to medium-term recurrence rates** in such selected cohorts. ([American Urological Association](#))

So: your patient’s PCP advice (“TRT increases recurrence risk”) reflects a **historically common and still-cautious approach**, but it is **no longer the only reasonable interpretation** for a man with **organ-confined disease, negative margins, and sustained undetectable PSA**—provided he accepts uncertainty about long-term oncologic safety.

Counseling: oncologic risk (what you can say honestly)

Best-supported statement:

In men who are **disease-free after prostatectomy** (undetectable PSA, favorable pathology), available **retrospective/observational data and pooled reviews generally have not shown an increased rate of biochemical recurrence** among those receiving TRT, but **randomized data and long-term safety data are limited**, especially for higher-risk pathology. ([PMC](#))

How that applies here:

- Grade Group 3 (4+3) is *intermediate risk*, not “lowest risk.” That doesn’t automatically preclude TRT, but it **pushes you toward more caution and tighter surveillance**.
- Many clinicians who offer TRT after prostatectomy use a threshold like:
 - **≥ 6–12 months post-op with undetectable/stable PSA**, and

- **no adverse pathology concerns** (or at least a careful discussion if present). (This “selected cases” approach is reflected in urology practice updates and reviews rather than definitive RCT-level evidence.) ([AUA News](#))

Bottom line framing for the patient:

TRT **may be reasonable** to consider given his sustained undetectable PSA at 12 months, but it should be treated as a **shared decision under uncertainty**: the best available evidence is reassuring in the short–medium term in selected men, but it is **not definitive**, and **long-term risk cannot be ruled out**. ([PMC](#))

Expected symptomatic benefits (and what’s realistic)

If his symptoms are truly due to testosterone deficiency, TRT can improve:

- **Energy/fatigue, libido, erectile function** (variable; ED after prostatectomy has multiple drivers)
- **Mood and sense of well-being** (modest average effects)
- **Lean mass/strength** (more consistent)
- **Bone density over time** (important if chronically low T)

Benefits usually appear over **weeks to months**, and not everyone responds—so it’s reasonable to set a **time-limited trial with objective symptom tracking**. ([OUP Academic](#))

Non-oncologic risks you should specifically review

Key risks/contraindications and monitoring points (standard TRT counseling):

- **Erythrocytosis/polycythemia** (check hematocrit; hold or adjust if high)
- **Worsening obstructive sleep apnea** (screen if symptoms)
- **Acne, edema, possible gynecomastia**
- **Infertility** (TRT suppresses spermatogenesis; less relevant at 59 but still worth stating)
- **Cardiovascular risk** remains debated; avoid starting immediately after major CV events and manage baseline risk factors carefully. ([OUP Academic](#))

A practical “if we do this” plan (shared decision + safety)

1) Confirm it's true hypogonadism and evaluate reversible causes

Before TRT, most clinicians document:

- Symptoms + **two low AM testosterone** (done)
- Consider **LH/FSH** (primary vs secondary), **prolactin** if secondary suspected, plus targeted evaluation for contributors (sleep deprivation/OSA, depression, thyroid disease, anemia, meds such as opioids/glucocorticoids, weight changes). ([OUP Academic](#))

2) Baseline cancer surveillance status (do not skip)

- **Baseline PSA (confirm undetectable immediately prior to start)** and ensure he remains on a standard post-prostatectomy surveillance pathway.
- Start TRT only with agreement that **PSA will be monitored closely** and TRT may be **stopped** if PSA behavior is concerning. ([AUA News](#))

3) Choose a conservative TRT approach

Common “risk-minimizing” practice patterns in this setting include:

- Prefer **short-acting, easily titratable formulations** (often **transdermal gel**) initially.
- Avoid supraphysiologic dosing; some urology guidance targets a **mid-normal range** rather than “high normal.” ([AUA News](#))

4) Monitoring schedule (what you'd tell him up front)

A reasonable monitoring framework (aligns with endocrine/urology monitoring principles):

- **PSA:** at baseline, then **every 3 months for the first year**, then spacing out if stable (e.g., every 6–12 months).
- **Testosterone level:** at ~6–8 weeks after initiation/dose change until in target range, then periodically.
- **Hematocrit/hemoglobin:** baseline, **3–6 months**, then at least annually (more often early on); adjust/hold if elevated.
- Also consider: lipids, LFTs per clinician preference; assess symptoms and adverse effects each visit. ([OUP Academic](#))

5) Clear “stop rules” (important for trust)

- If PSA becomes detectable and shows a **confirmed rise** (pattern concerning for biochemical recurrence), **stop TRT and refer back to urology/oncology** for recurrence workup and management. ([ScienceDirect](#))

How I'd summarize to the patient (plain language)

“You’re one year out from surgery with negative margins and an undetectable PSA, which is reassuring. Historically, testosterone was avoided after prostate cancer, and some guidelines still take a conservative stance. However, newer observational data in carefully selected men like you generally hasn’t shown higher recurrence rates in the short to medium term, though we don’t have perfect long-term randomized data. If we proceed, we should do it with your urologist involved, aim for mid-normal testosterone (not high), and monitor PSA very closely with clear agreement that we would stop therapy if PSA trends are concerning.” ([OUP Academic](#))

If you want, I can also draft a one-paragraph “assessment & plan” in clinician style (problem list + counseling + monitoring orders) that you could paste into a note.